

1. Administrative & Study Identification

Full Study Title: Phase II/III study for evaluation of the diagnostic performance of [18F]CTT1057 PET imaging for the detection of PSMA positive tumors using histopathology as a standard of truth (GuideView) **Short Title/Acronym:** GuideView
Study Protocol Number: CAAA405A12302 **NCT Number:** NCT04838626
Sponsor Name: Novartis **Investigational Product:** [18F]CTT1057 (vidoflufolastat (18F)) **Phase of Development:** Phase II/III
Medical Monitor: Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the diagnostic performance (sensitivity and specificity) of [18F]CTT1057 as a positron emission tomography (PET) imaging agent for the detection and localization of Prostate Specific Membrane Antigen (PSMA) positive tumors, using histopathology as the Standard of Truth (SoT). **Secondary Objectives:** To evaluate the positive predictive value (PPV), negative predictive value (NPV), correct localization rate, and overall safety and tolerability of the imaging agent.

2.2 Background & Rationale To address the clinical need for accurate staging and localization of primary tumors and pelvic lymph node metastases in newly diagnosed high-risk prostate cancer (PCa) patients. PSMA is a transmembrane glycoprotein overexpressed in approximately 90% of prostate cancer cells. Accurate baseline staging using PSMA-targeted PET imaging helps guide surgical and therapeutic management, potentially improving prognostic outcomes by identifying patients most likely to benefit from targeted interventions.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Diagnostic Imaging) **Control Type:** Single-arm **Blinding:** Open-label (with blinded central reading of imaging) **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 195 participants enrolled (to ensure at least 156 evaluable patients) **Number of Sites:** Multi-center (Global, including US and Spain) **Estimated Study Start:** April 2021 **Estimated Primary Completion:** Completed

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults ≥ 18 years of age. 2. Untreated high-risk biopsy-proven PCa patients according to D'Amico classification (Stage $\geq T2c$ or PSA level >20 ng/ml or Gleason score ≥ 8). 3. Scheduled or planned radical prostatectomy and extended pelvic lymph node resection up to 6 weeks after the investigational PET/CT scan, followed by histopathology assessment.

4.2 Exclusion Criteria 1. Prior and current use of PSMA targeted therapies. 2. Prior and current treatment with any Androgen Deprivation Therapy (ADT, first or second generation), including LHRH analogues, or 5-alpha reductase inhibitors within 30 days before screening. 3. Patients with small cell or neuroendocrine PCa in more than 50% of biopsy tissue. 4. Severe urinary incontinence, hydronephrosis, severe voiding dysfunction, or New York Heart Association (NYHA) class III or IV congestive heart failure.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Single diagnostic dose of [18F]CTT1057 (vidoflufolastat (18F)) prior to image acquisition. **Route of Administration:** Intravenous (IV) injection. **Duration of Treatment:** Single PET/CT imaging session, followed by surgery up to 6 weeks post-scan.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care routines for prostate cancer preoperative preparation. **Prohibited:** ADT, PSMA-targeted therapies, 5-alpha reductase inhibitors, and other investigational drugs within 30 days prior to screening.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -30 to 0	Informed Consent, Medical History, ECOG assessment, confirming high-risk PCa eligibility
Baseline	Day 1	Administration of [18F]CTT1057, PET/CT scan acquisition, Safety Monitoring
Surgery	Up to 6 Weeks Post-Scan	Radical prostatectomy, extended pelvic lymph node resection, tissue collection for histopathology (SoT)
End of Study	Post-Surgery	Final Safety evaluation, AE review, Exit Interview

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Diagnostic performance of [18F]CTT1057 PET imaging, specifically patient-level sensitivity

and region-level specificity for the detection of PSMA-positive lesions. **Measurement Method:** Concordance between [18F]CTT1057 PET/CT imaging findings and histopathology results (Standard of Truth) derived from resected primary tumor and pelvic lymph node tissues.

7.2 Secondary & Exploratory Endpoints * Positive Predictive Value (PPV) and Negative Predictive Value (NPV). * Correct localization rate of pelvic lymph node metastases. * Assessment of safety and tolerability of [18F]CTT1057.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection from the time of radiotracer injection through the postoperative follow-up visit. **Serious Adverse Events (SAEs):** Reported within standard 24-hour timelines from investigator awareness. **Data Monitoring Committee (DMC):** Internal safety and pharmacovigilance committee managed by Novartis.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Evaluable Population consisting of patients who successfully undergo the PET/CT scan, proceed to surgery without receiving prohibited intermediate therapies, and have evaluable histopathology. **Sample Size Justification:** A total enrollment of 195 patients was required to ensure at least 156 evaluable subjects to adequately power the primary sensitivity and specificity calculations against predefined success thresholds. **Handling of Missing Data:** Patients missing the Standard of Truth (histopathology) due to canceled surgeries or unreadable scans were excluded from the primary efficacy evaluable set.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Clinical Practice (GCP) guidelines and the Declaration of Helsinki. **Monitoring Plan:** Continuous site monitoring to ensure protocol adherence and strict tracking of imaging-to-surgery timelines. **Audit Trail:** Secure eCRF systems, centralized independent and blinded readings of the PET/CT scans, and standardized central pathology reviews to ensure unbiased endpoint analysis.

11. Study Identifiers & Governance

Primary ID: CAAA405A12302 **Registry Number:** NCT04838626 **Sponsor/Lead Organization:** Novartis **Study Title:** GuideView **Official Regulatory Title:** Phase II/III study for evaluation of the diagnostic performance of [18F]CTT1057 PET imaging for the detection of PSMA positive tumors using histopathology as a standard of truth (GuideView)

12. Clinical Framework (Prostate Cancer Imaging Specific)

Primary Condition: High-Risk Prostate Cancer (PCa) **Diagnosis Threshold:** Stage $\geq$ T2c, PSA level > 20 ng/ml, or Gleason score ≥ 8 **Medical Methodology:** PSMA-targeted Positron Emission Tomography (PET) Imaging **Optimization Target:** Accurate preoperative localization and staging of primary tumors and pelvic lymph node metastases

13. Study Procedures & Methodology

Management Pathway: Diagnostic PET/CT Scan followed by Radical Prostatectomy **Education Protocol (HCP):** Site qualification on radioactive tracer handling, PET/CT acquisition protocols, and standardized surgical resection techniques **Education Protocol (Patient):** Informed consent detailing the use of radioactive imaging agents and surgical follow-up **Quality Audit Mechanism:** Independent blinded central imaging readers and standard histopathology verification (SoT)

14. Observation & Follow-Up Schedule

Total Duration: Up to approx. 8-10 weeks (Screening through Post-Surgical Follow-up) **Onsite Visit Cadence:** Screening, Baseline (Imaging Day), Surgery (Within 6 Weeks), and Post-Op Follow-up **Remote Monitoring Frequency:** Intermittent remote tracking for AE reporting post-injection **Checklist Review Interval:** Final reconciliation upon histopathology readout

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				